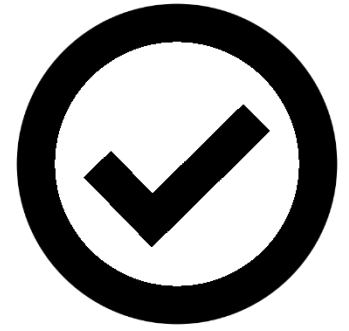




**BUSINESS  
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# **HEALTH ADMINISTRATION PROCEDURES (610) STATE 2021**

**Questions:**

**Objective Questions** \_\_\_\_\_ (80 points)  
*40 @ 2 points each*

**Production:**

**Job 1: HPIP** \_\_\_\_\_ (100 points)

**Job 2: SOAP** \_\_\_\_\_ (100 points)

**TOTAL POINTS:** \_\_\_\_\_ (280 points)

**Test Time: 60 minutes**



|             |             |
|-------------|-------------|
| <b>1. B</b> | <b>21.D</b> |
| <b>2. A</b> | <b>22.B</b> |
| <b>3. B</b> | <b>23.C</b> |
| <b>4. A</b> | <b>24.A</b> |
| <b>5. D</b> | <b>25.A</b> |
| <b>6. C</b> | <b>26.C</b> |
| <b>7. A</b> | <b>27.C</b> |
| <b>8. C</b> | <b>28.A</b> |
| <b>9. C</b> | <b>29.C</b> |
| <b>10.D</b> | <b>30.C</b> |
| <b>11.D</b> | <b>31.A</b> |
| <b>12.A</b> | <b>32.D</b> |
| <b>13.B</b> | <b>33.C</b> |
| <b>14.D</b> | <b>34.A</b> |
| <b>15.D</b> | <b>35.A</b> |
| <b>16.C</b> | <b>36.B</b> |
| <b>17.B</b> | <b>37.C</b> |
| <b>18.B</b> | <b>38.C</b> |
| <b>19.D</b> | <b>39.B</b> |
| <b>20.A</b> | <b>40.A</b> |

**HPIP Medical Transcription Form**

Top Margin: 1"

Side Margins: 1"

Patient Name Line: 2.5" from top

Second Page Text begins: 1" from top



**Job 1 – HPIP Medical Transcription Form**

Patient Name: Casey Smith

PCP: Linda Thompson, M.D.

Date of Birth: 01/24/1954

Date of Exam: *(Today's Date)*

**HISTORY OF PRESENT ILLNESS:** This is a 67-year-old male with a history of a left vocal cord cancer, which was treated with radiation therapy. He subsequently had recurrence, which resulted in hemi laryngectomy. After that, he had another primary requiring total laryngectomy. Then he developed a radiation-induced sarcoma of the nasopharynx and, therefore, underwent resection of this with reconstruction with a pectoralis flap. His postoperative course was complicated by necrosis and fistula, with the requirement of being taken back to the operating room for a second operation last month for debridement and reclosure of the wound.

Today he is here for follow-up. He recently underwent an esophageal dilation. He reports a 40% improvement in his swallowing since that time. He reports dysphagia for solids and liquids, which has been worsening since his dilation. He denies any dyspnea, odynophagia, otalgia, weight loss, or any other complaints. Past medical history, past surgical history, medications, allergies, family history, social history, and review of systems were reviewed and found to be unchanged from the previous note.

**MEDICATIONS:** Coumadin, Lanoxin, diltiazem, furosemide, pentoxifylline, Zocor, and lisinopril.

**PHYSICAL EXAMINATION:** Vital signs are as follows: Temperature 96.9, pulse 57, respiratory rate 16, and weight 154 pounds, blood pressure 104/50. In general, the patient is a well-nourished male in no apparent distress, alert and oriented  $\times 3$ . Voice is with an electrolarynx. Head: No scalp or facial lesions or asymmetry. Eyes: Extraocular movements intact. Anicteric. Ears: TMs intact with no middle ear effusions. Nose: No masses. No discharge. Septum is straight. Oral cavity/oropharynx: Mucous membranes are moist. By visualization and palpation, no masses or lesions are seen including cheeks, floor of mouth, hard and soft palate, tongue, tonsils, posterior pharynx, lips, teeth, and gums. Indirect laryngoscopy: No masses within the nasopharynx, and the pectoralis flap is visible and viable. Neck: Laryngectomy stoma is widely patent without any lesions. There are no neck masses or lymphadenopathy. Incisions are all well healed.

**IMPRESSION:** No evidence of disease at this time. Patient does have worsening dysphagia and was offered esophageal dilation whenever he desires it. He does not want any at this time and would like to return to clinic in 3 months.

**PLAN:** Return to clinic in 3 months.

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Linda Thompson, M.D.

LT: Member ID

D: (Today's Date)

T: (Tomorrow's Date)



## JOB 2 – SOAP Medical Transcription Form

Patient Name: Scott Staples

PCP: Drake Ramoray, M.D.

Date of Birth: 10/20/1943

Date of Exam: (Today's Date)

Sex: Male

**SUBJECTIVE:** The **patient** returns today for evaluation of an abnormal chest x-ray. The patient had been in the emergency room 1 week ago with a complaint of right pleuritic chest pain. At that time, the chest x-ray was read as possible respiratory infection. Chest x-ray was obtained 1 year ago and revealed hilar granulomas and no follow-up was obtained. The patient was placed on erythromycin and Vicodin in the emergency room. He states that **he** has marked improvement in his pleuritic chest pain. He no longer has productive cough. He has been a smoker for many years and is currently smoking one-half pack of cigarettes per day. He worked as a janitor for many years. He denies any knowledge of severe pulmonary infections in the past.

**OBJECTIVE:** The patient appears to be generally healthy. He is slightly overweight but well-muscled. **VITAL SIGNS:** Blood pressure 130/84, weight 188 pounds. **SKIN:** Acne over the face and psoriasis of the legs, both moderately active. **NECK:** There are no cervical or supraclavicular nodes palpable. **LUNGS:** Clear to auscultation and percussion. **HEART:** S1 and S2, no murmurs, rubs, or gallops.

**ASSESSMENT:** 1. Recent acute lower respiratory infection with pleurisy.  
2. Abnormal chest x-ray with granulomas.  
3. Tobacco abuse.  
4. Multiple other systemic complaints as above.

**PLAN:** Discussed with the patient the nature of coccidioidomycosis and explained that an abnormal chest x-ray does not equal the diagnosis of cocci, but this certainly should be pursued. The patient will return in 1 week for follow-up.

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Drake Ramoray, M.D.

DR: Member ID

D: (Today's Date)

T: (Tomorrow's Date)